BASELINE → TREATMENT → FOLLOW-UP

Tests and checks

Required safety blood tests are separate from the individually prescribed scan or tumour-marker plan.

Check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver	Baseline tests and dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) status	Weekly throughout treatment	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up
Radiotherapy / pump	Radiation planning; line and pump training	Weekly bloods and pump changes; separate radiation visits	Report bowel, bladder, skin or line problems	Assess nutrition and hydration	Radiation/colorectal team decides response assessment and next steps

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00421 v6 official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Continuous fluorouracil + radiotherapy: treatment and checks

Locally advanced rectal cancer

Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver NCCP 00421	Baseline tests and DPD status	Weekly throughout treatment	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines NCCP 00421	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers NCCP 00421	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up
Radiotherapy / pump NCCP 00421	Radiation planning; line and pump training	Weekly bloods and pump changes; separate radiation visits	Report bowel, bladder, skin or line problems	Assess nutrition and hydration	Radiation/colorectal team decides response assessment and next steps

What each check is for

1 Confirm your personal plan

Before starting

Continuous fluorouracil helps radiotherapy treat locally advanced rectal cancer. Your team explains how this fits with surgery and other treatment.

Ask / **act:** Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00421

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32